

1. Administrative & Study Identification

Full Study Title: A Phase 1/2 Study of BMS-986253 in Combination With Nivolumab or Nivolumab Plus Ipilimumab in Advanced Cancers
Short Title/Acronym: A Study of BMS-986253 in Combination With Nivolumab or Nivolumab Plus Ipilimumab in Advanced Cancers
Protocol Number: CA027002 **NCT Number:** NCT03400332 **Posting ID:** 678672725820664972 **Sponsor/Company Name:** Bristol Myers Squibb
Trial Status: Active but not currently recruiting (ACTIVE_NOT_RECRUITING) **Investigational Product:** BMS-986253 (anti-IL-8 monoclonal antibody), Nivolumab, Ipilimumab (Trade Name: Yervoy, ATC Code: L01FX04, EMA: [Product Information](#)) **Phase of Development:** Phase 1/Phase 2 **Medical Monitor:** Study Director/ Medical Monitor (Bristol Myers Squibb)

2. Study Synopsis & Rationale

2.1 Study Objective **Primary Objective:** To investigate the safety, tolerability, and objective response rate of the experimental medication BMS-986253 (anti-IL-8) in combination with Nivolumab or Nivolumab plus Ipilimumab in participants with advanced cancers (including advanced malignant melanoma). **Secondary Objectives:** To evaluate Progression-Free Survival (PFS), Overall Survival (OS), Duration of Response (DOR), Complete Response (CR), Partial Response (PR), and the pharmacokinetics (PK) and pharmacodynamics of the combination therapies.

2.2 Background & Rationale Tumor cells evade immune recognition due to the immunosuppressive features of the tumor microenvironment. Interleukin-8 (IL-8) drives key pathological processes such as epithelial-mesenchymal transition (EMT), angiogenesis, and the recruitment of immunosuppressive polymorphonuclear myeloid-derived suppressor cells (MDSCs). Blocking the IL-8 axis with the targeted antibody BMS-986253 may reverse these immunosuppressive effects, making tumors less aggressive and more responsive to immune checkpoint inhibitors (Nivolumab and Ipilimumab).

3. Study Design & Methodology

3.1 Design Framework **Study Type:** Interventional **Control Type:** Placebo-controlled (in Part 2, comparing BMS-986253 vs. Placebo combined with Nivolumab + Ipilimumab), and Dose-escalation/ Single-arm (in Part 1). **Blinding:** Double-blind (Part 2); Open-label (Part 1) **Randomization:** Randomized (Part 2)

3.2 Planned Enrollment & Duration Targeted Enrollment (\$N\$): 281 participants. **Number of Sites:** Multicenter global study **Estimated Study Start:** 12-Feb-2018 **Estimated Primary Completion:** 26-Apr-2024 (Actual)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Histologic or cytologic confirmation of a solid tumor that is advanced (metastatic, recurrent and/or unresectable) with measurable disease per RECIST v1.1. 2. At least 1 lesion accessible for biopsy. 3. Eastern Cooperative Oncology Group Performance Status of 0 or 1.

4.2 Exclusion Criteria 1. Participants with CNS metastases as the only site of active disease (Participants with controlled brain metastases; however, will be allowed to enroll). 2. Participants with active, known or suspected autoimmune disease. 3. Participants with conditions requiring systemic treatment with either corticosteroids (> 10mg prednisone equivalents) or other immunosuppressive medications within 14 days of study treatment administration. 4. Participants with a known history of testing positive for Human Immunodeficiency Virus (HIV) or known Acquired Immunodeficiency Syndrome (AIDS). 5. Cytotoxic agents, unless at least 4 weeks have elapsed from last dose of prior anti-cancer therapy and initiation of study therapy.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: BMS-986253 administered in flat doses (e.g., 600 mg, 1200 mg, 2400 mg, or 3600 mg) every 2 weeks (Q2W) or every 4 weeks (Q4W), combined with Nivolumab (e.g., 480 mg Q4W) \$\\pm\$ Ipilimumab. **Route of Administration:** Intravenous (IV) infusion. **Duration of Treatment:** Maximum treatment duration of up to 3 years, or until disease progression or unacceptable toxicity.

5.2 Concomitant & Prohibited Therapy Permitted: Palliative radiation therapy for non-CNS disease (provided a 1-week washout period is observed) and standard concomitant medications not affecting immune status. **Prohibited:** Systemic corticosteroids (>10 mg prednisone equivalent) within 14 days of study treatment, cytotoxic therapies within 4 weeks, and concurrent investigational treatments.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, Tumor Biopsy, ECOG Assessment, baseline sIL-8
Baseline	Day 0	Randomization (Part 2) or Cohort Assignment	

(Part 1), First Dose Administration | | **Interim** | Every 2-4 Weeks
| Safety Labs, Efficacy Assessment (RECIST v1.1 scans), PK/PD
sampling (Cmax, AUC), AE tracking | | **End of Study** | Up to Year 3
| Treatment Discontinuation, Final Scans, Survival Follow-up |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints * **Number of Participants Experiencing Adverse Events (AEs)** - **Part 1:** An Adverse Event (AE) is defined as any new untoward medical occurrence or worsening of a pre-existing medical condition in a clinical investigation participant administered study treatment and that does not necessarily have a causal relationship with this treatment. AEs are graded on a scale from 1 to 5 (1: mild and asymptomatic; 2: moderate; 3: severe or medically significant; 4: severe requiring hospitalization; 5: fatal). Pre-specified for Part 1. * **Number of Participants Experiencing Serious Adverse Events (SAEs)** - **Part 1:** Any untoward medical occurrence that results in death, is life-threatening, requires inpatient hospitalization, results in persistent or significant disability/incapacity, is a congenital anomaly/birth defect, or is an important medical event. Pre-specified for Part 1. * **Number of Participants Experiencing Dose Limiting Toxicities (DLTs)** - **Part 1:** DLTs are effects of a treatment that are serious enough to prevent an increase in dose of that treatment. DLTs will be defined based on the incidence, intensity, and duration of AEs that are possibly related to study treatment (including gastrointestinal, hepatic, hematologic, dermatologic). Pre-specified for Part 1.

7.2 Secondary & Exploratory Endpoints * Objective Response Rate (ORR) - Part 1 * Duration of Response (DOR) - Part 1 * Maximum Concentration (Cmax) - Part 1 * Progression-Free Survival (PFS) * Overall Survival (OS) * Complete Response (CR) and Partial Response (PR)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: AEs and laboratory abnormalities are monitored and graded according to the Common Terminology Criteria for Adverse Events (CTCAE) version 5.0. **Serious Adverse Events (SAEs):** Tracked continuously, including SAEs collected during the Pre-Screen period related to biopsy or blood draws, followed until resolution or stabilization. **Data Monitoring Committee (DMC):** Periodic safety and dose-escalation reviews ensure tolerability profiles are maintained before escalating doses or transitioning to subsequent study parts.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: All randomized participants (modified in protocol amendment to remove restriction by sIL-8 baseline status). **Sample Size Justification:** The targeted enrollment of 281 was statistically sized to sufficiently evaluate the endpoints of safety and Objective Response Rate (ORR) under a randomized, double-blind framework (Part 2) and single-arm escalation (Part 1). **Handling of Missing Data:** Standard oncological statistical imputation (e.g., Kaplan-Meier methods for time-to-event endpoints like PFS/OS/DOR).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine clinical monitoring for data accuracy, safety reviews, and protocol adherence. **Audit Trail:** Secure database entry, electronic case report forms (eCRF), and independent central review mechanisms for all imaging endpoints.

11. Study Identifiers & Governance

Primary ID: CA027002 **Registry Number:** NCT03400332 **Posting ID:** 678672725820664972 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** A Study of BMS-986253 in Combination With Nivolumab or Nivolumab Plus Ipilimumab in Advanced Cancers **Official Regulatory Title:** A Phase 1/2 Study of BMS-986253 in Combination With Nivolumab or Nivolumab Plus Ipilimumab in Advanced Cancers

12. Clinical Framework (Oncology/IL-8 Specific)

Disease Areas: Cancer, Melanoma **Primary Condition:** Advanced Solid Tumors (Cancer), Melanoma / Advanced malignant melanoma **Condition Classifications:** Neoplastic Process (MeSH: D008545; HPO: HP: 0002861; SNOMED: 372244006) **Diagnosis Threshold:** Measurable disease per RECIST v1.1 **Medical Methodology:** Combined immune checkpoint inhibition with tumor microenvironment (TME) modulation **Optimization Target:** Neutralization of IL-8 to reverse myeloid-derived immunosuppression and enhance PD-1/CTLA-4 therapeutic efficacy

13. Study Procedures & Methodology

Management Pathway: Interventional Oncology Care Pathway **Treatment Protocol (HCP):** Infusion center administration of targeted monoclonal antibody (BMS-986253) alongside established

immunotherapy (Nivolumab \$\pm\$ Ipilimumab) **Observation Protocol:** Standard serial oncologic imaging and immune-related adverse event (irAE) monitoring **Quality Audit Mechanism:** Blinded Independent Central Review (BICR) for tumor response measurements

14. Observation & Follow-Up Schedule

Total Duration: Up to 3 years of treatment + survival follow-up
Onsite Visit Cadence: Matched to infusion cycles (Q2W or Q4W)
Radiological Monitoring Frequency: Periodic scanning (e.g., every 8-12 weeks) mapped to RECIST v1.1 criteria evaluation **Safety Review Interval:** Continuous AE monitoring during the active phase and standard follow-up intervals post-treatment discontinuation

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				